Patient Name: Wendy Santos

DOB: 11/13/1989

Reason for referral: abdominal pain

Patient phone: 530-555-2016

Primary insurance: Medicare

SUMMIT PACIFIC
• MEDICAL CENTER •

Patient Name: Wendy Santos
DOB: 11/13/1989
Phone: 360-495-5780
Reason for referral: abdominal pain
Patient phone: 530-555-2016
Primary insurance: Medicare

Endoscopy Procedure Referral

Fax: 360-495-5781

Patient Information

Height _____ Weight _____ BMI _____

Patient last name: _____ First: _____ MI ___ DOB: _____ Age: ____ Sex: _____

Patient Address: _____ City/State: _____ Zip: _____

Home #: _____ Work #: _____Cell#: _____ Email: _____

Referral Information

Referring Provider: _____ Location: _____ Office Phone: _____

Contact Name: _____ Office Fax: _____

Diagnosis: _____ ICD 10: _____

Primary Insurance: _____ Coverage eligibility phone: _____

Member ID #: _____ Group #: _____ Effective Date: _____

Colonoscopy

- ☐ Screening: 45 years or older with average age risk

Specific indications:

- ☐ Personal hx of polyps. Type: _____
  - Colonoscopy date: _____
- ☐ Personal hx of colorectal cancer
  - Last colonoscopy date: _____
- ☐ Personal hx of inflammatory bowel disease
- ☐ Family hx of colorectal cancer or polyps
  - Relation _____ Age at dx___
  - Relation _____ Age at dx___
- ☐ Fecal occult blood positive, date _____
- ☐ Iron deficiency
- ☐ Hematochezia (rectal bleeding)
- ☐ Other, describe _____

Flexible Sigmoidoscopy

- ☐ Screening
- ☐ Suspected rectal disease when colonoscopy is not indicated

EGD (Upper Endoscopy)

- ☐ Upper abdominal distress/dyspepsia
- ☐ Dysphagia/Odynophagia (circle one)
- ☐ Nausea/vomiting
- ☐ Gastric ulcer
- ☐ Hiatal Hernia
- ☐ Gastrointestinal bleed/iron deficiency with suspected upper GI source
- ☐ Barrett's esophagus surveillance
  - Date of last EGD _____

****Submission *must* include the following information:**

- ☐ **Patient face sheet**
- ☐ **List of current medications**
- ☐ **Surgical and medical history**
- ☐ **Recent history and physical**
- ☐ **Endoscopy Procedure reports if applicable**

ClinLab
Clinical Testing Laboratory, Inc.

Clinical Testing Lab of California
55492 Spring Street, Suite 201
Vienna, CA 94078-9932
Phone: 510-555-1212

Specimen Number	Patient ID	Control Number	Account Number	Account Phone Number	Route

Patient Last Name	Account Address

Patient First Name	Patient Middle Name
Jeff	

Patient SS#	Patient Phone	Total Volume

Age (Y/M/D)	Date of Birth	Sex	Fasting
46 v.o.		Male	Yes

Patient Address	Additional Information

Date and Time Collected	Date Entered	Date and Time Reported	Physician Name	NPI	Physician ID

Tests Ordered
CBC, Platelet Ct, and Diff ; Liver Function (AST/ALT) ; Iron/Iron Binding Panel

General Comments

TESTS	RESULT	FLAG	UNITS	REFERENCE INTERVAL	LAB
CBC, Platelet Ct, and Diff					
Hematocrit	40.6		%	40.8 - 51.9	07
Hemoglobin	14.2		g/dL	14.6 - 17.8	07
Red Blood Cell Count	8.4		x10E6/uL	4.69 - 6.07	07
White Blood Cell Count	7.4		x10E3/uL	3.2 - 10.6	07
RDW	13.0		%	10.8 - 14.1	07
MCV	82.6		fL	77.8 - 94.0	07
MCH	27.4		pg	26.5 - 32.6	07
MCHC	34.4		g/dL	32.7 - 36.9	07
Imm.Granulocytes (Absolute)	6.2		x10E3uL	1.3 - 7.0	07
Granulocytes (Percent)	57.2		%	44.0 - 76.0	07
Eosinophils (Absolute)	0.3		x10E3uL	0.0 - 0.4	07
Eosinophils (Percent)	5.2		%	0.0 - 6.0	07
Basophil (Absolute)	0.08		x10E3uL	0.0 - 0.1	07
Basophil (Percent)	1.7		%	0 - 2	07
Monocytes (Absolute)	0.64		x10E3uL	0.2 - 0.7	07
Monocytes (Percent)	6.9		%	4 - 8.9	07
Lymphocytes (Absolute)	2.26		x10E3uL	0.8 - 3.1	07
Lymphocytes (Percent)	37.2		%	15 - 43	07
Platelets (Absolute)	388		x10E3uL	177 - 406	07
Liver Function					
Aspartate aminotransferase (AST)	223	High	U/L	8 - 43	07
Alanine Aminotransferase (ALT)	257	High	U/L	7 - 45	07
Iron and Total Iron Binding					
Serum Iron	2300	Very High	ug/dL	40 - 155	07
Total Iron Binding Capacity	328		ug/dL	250 - 400	07
Transferrin Saturation	72	Very High	%	20 - 50	07

PRELIMINARY INTERPRETATION:

A preliminary diagnosis of Hemochromatosis can be made based on very high Serum Iron and Transferrin Saturation values. Elevated liver enzyme levels are indicative of liver failure which is a common consequence of Hemochromatosis.

There are well established pathogenic genetic variants that are strongly correlated with Hereditary hemochromatosis. The presence of a hereditary disorder has consequences for family members. Genetic testing and counseling should be considered by the patient and family members.